

Sample question from
previous coc

- 1.a 5 year old boy with HIV infection on art medication presented with progressive difficulty of breathing for three weeks duration has no history of fever ,on examination pR 108/min RR 72/min T 37.2 spo2 70% chest radiography reveals bilateral alveolar disease and with cd4 count of less than 50 what is the most likely diagnosis

A,heart failure

B.Pneumocystis pneumonia

c.Pulmonary TB

D.Bacterial pneumonia

- 2. which of the following is NOT expected in child with history of fever, headache, neck stiffness in CSF analysis

A. Decrease glucose level

b. Leukocytosis

c. Increased protein level

d. None

- A 6 month old infant presented with cough and fast breathing of three days duration. Further inquiry reveals history of repeated chest infections, interruption during breast feeding and poor weight gain. Precardial examination reveals active precardium and pansystolic murmur at left lower sternal border. Which of the following most likely explains those findings?
- A. Atrial septal defect
- B. ventricular septal defect
- C. patent ductus arteriosus
- D. Tetralogy of fallot

- An 8- year-old boy known cardiac patient developed lowgrade fevwr, fatigue myalgia in the last two weeks. On examination he was found to have developed a new murmur, petechiae rashes and mild splenomegaly, which one is the most likely diagnosis?
- A. Acute rheumatic fever
 - B. Dilated cardiomyopathy
 - C. infective endocarditis
 - D. Rheumatic heart disease

A 14 –month old male child is brought to the ER with watery diarrhea of 24 hours duration. Physical examination reveals irritable infant, weight= 12 kg. sunken eyeballs and eagerness to drink. What is the correct type and volume of fluid for this infant?

- A. Ringer's lactate, 300ml
- B. 0.4% Normal saline, 700ml
- C. oral rehydration solution, 900ml
- D. Ringer's lactate, 500ml then ORS 500ml

which of the following clinical conditions that appear after pertussis vaccination is an absolute contraindication to subsequent administration of the vaccine?

- A. Fever and convulsion within 3 days
- B. Immediate anaphylactic reaction
- C. inconsolable cry longer than 2 hours
- D. shock-like state after 2 days

you are evaluating an HIV exposed neonate immediately after delivery. The mother had proper ANC follow up and was managed according PMTCT option B+. which of the following is an appropriate recommendation for this new born?

- A. Avoid breast feeding
- B. Cotrimoxazole prophylaxis in the first weeks of life
- C. DNA-PCR test within 48 hours
- D. nevirapine for 6 weeks

A 3 month old infant has rhinorrhea cough and noisy breath. Physical examination reveals RR=72/min PR=142/min and T=37.80c he is irritable and has diffuse wheezes and chest • retraction. What is the most likely diagnosis?

A. Bronchiolitis

B. Gastro-esophageal reflux disease

D. severe pneumonia

. The recommended time to start folic acid supplementation for a mother who previously had a baby boy with myelomeningocele to prevent its recurrence in the next pregnancy is

- A. At the first missed period
- B. End of first trimester
- C. one month before conception
- D. third trimester

- which of the following is the correct percentage of dextrose to treat neonatal hypoglycemia?

A. 5%

B. 10%

C. 40%

D. 50%

. A 36 month-old male infant presented with high grade fever and vomiting of two days duration. He has right costovertebral angle tenderness. White cell count showed leukocytosis with left shift and blood film is negative for hemoparasites. What is the most likely diagnosis in this child?

- A. Pyelonephritis
- B. Hepatitis
- C. Appendicitis
- D. Lobar pneumonia

. A 2 year-old child has delay in all developmental mile stones for his age. On examination he has macrocephaly, severe stunting , protruded tongue. Umbilical hernia and coarse facial features. The best initial test is to determine serum level of;

- A. cortisol
- B. Growth Hormone
- C. Thyroid stimulating hormone
- D. Vitamin D

Which one of the following drugs is an appropriate antidote malathion poisoning?

- A. Atropine
- B. Flumazenil
- C. N-acetyl cysteine
- D. Naloxone

A 3 year-old boy presented with a complaint of intense pruritis worse at night. On examination there are vesicular and popular eruptions at the interdigital spaces. Which one is the best treatment for this child? Dx scabies

- A. Acyclovir
- B. Erythromycin
- C. Mebendazole
- D. Permethrin

- 0. A 2-hours term neonate is delivered by elective C/C for placenta previa. On physical examination PR=170/min, RR=72/min and oxygen saturation of 80%. What is the most likely cause of respiratory distress in this neonate?
 - A. Congenital pneumonia
 - B. Perinatal Asphyxia
 - C. Respiratory distress syndrome
 - D. Transient tachypnea of the new born

A 2 year- old child presented with difficulty of breathing of two hours. He had a breaking cough and hoarseness of voice for the last two days. On physical exam he is agitated, has audible stridor at rest and lower chest indrawing. What is the most likely diagnosis ?

- A. Croup syndrome
- B. Foreign body aspiration
- C. Pertussis
- D. Severe pneumonia

A 7 year old female patient who is on follow-up for bronchial asthma presented with shortness of breath of three hours. On physical examination she is conscious, able to talk in complete sentence, RR=62/min and PR=110/min. chest examination reveals diffuse expiratory wheeze. What is the initial best step in the management of this patient?

- A. Aminophylline
- B. Magnesium sulphate
- C. Oral prednisolone Epinephrine
- D. salbutamol puff

A 13- year old street boy presented to the ER with high grade fever of 5 days. He has epistaxis, fatigue and headache. On physical examination heart rate 120/min, respiratory rate 32/min, T=39.0

°C and has petechial rash on trunk. Which of the following is the best initial test for diagnosis?

- A. Blood film
- B. complete blood count
- C. CSF analysis
- D. widal test

An 18-months old child with severe acute malnutrition is noticed to have diffuse left thigh swelling which is erythematous, hot and tender to touch. It is also fluctuating. Which one of the following is the most likely diagnosis?

- A. Cellulitis
- B. Hip arthritis
- C. Osteomyelitis
- D. pyomyositis

- A health officer was attending delivery of a post term pregnancy. Upon follow-up of the labor there was a record of fetal tachycardia and grade III meconium stained amniotic fluid. The newborn then failed to breathe spontaneously immediately after delivery. What is the most likely cause of respiratory problem in this baby?
 - A. Respiratory distress syndrome
 - B. Congenital pneumonia
 - C. Meconium aspiration syndrome
 - D. Congenital heart disease

A 5 year old child is diagnosed with Group A beta hemolytic tonsillo-pharyngitis. Which one is a non – suppurative complication of this condition?

- A. Otitis media
- B. Peritonsillar abscess
- C. Retropharyngeal abscess
- D. Rheumatic fever

A 3year old boy came to the OPD with diarrhea. Stool examination revealed trophozoite of Gardia lambia. What is the first line drug to treat this parasite?

- A. Tinidazole
- B. Metronidazole
- C. Paromomycin
- D. Albendazole

- An 18 month old child brought with a complaint of intractable vomiting of ingested matter & discomfort in his neck since two days ago. Chest x-ray showed that there is a circular opaque lesion behind the trachea. What is the most likely Dx of this child?
- | | |
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| A) Foreign body aspiration | C) Esophageal stenosis |
| B) Foreign body ingestion | D) Esophageal tear |

34. A three week infant born at 32 weeks of gestation & weighing 1750g, was brought to OPD with complaint of abdominal distension, feeding intolerance, and vomiting. On examination, the infant is lethargic with grossly distended & tender abdomen. Laboratory investigation shows leucopenia & thrombocytopenia. Plain abdomen x-ray shows air in the bowel wall. What is the most appropriate immediate step in the management of the infant?

- A) NPO with IV fluids
- B) Systemic antibiotic

- C) Surgical consultation
- D) Nasogastric decompression

A four day infant, delivered at home, was presented with complaints of irritability & difficulty in opening of the mouth associated with hunger & crying. Physical examination revealed tachypnea, tachycardia, fluctuating temperature & severe spasms with stimulation. What measure could have prevented this condition from occurring?

- A) Active immunization of the neonate with DPT
- C) Human tetanus immunoglobulin
- B) Maternal immunization with tetanus toxoid
- D) Prophylactic antibiotics at birth

- 36. A neonate born to a diabetic mother, was brought to the clinic with irritability & jitteriness. It is large, plethoric & tretrulous. Physical examination reveals tachycardia, tachypnea & hypothermia. What is the most likely explanation for this condition?
 - A) Hyperglycemia
 - B) Hypercalcemia
 - C) Hypoglycemia
 - D) Polycythemia

- A one year old infant was brought to the clinic with complaint of low grade fever and runny nose followed by paroxysm of whooping cough of two weeks duration. He has shortness of breath & becomes restless during the episodes of cough, and vomits following the paroxysms of cough. The mother says that the infant has not been vaccinated. Which vaccine could have prevented this condition?

A) BCG B) OPV C) DPT D) Vitamin A

- A mother brought her two yr old child with complaints of coryza, cough, conjunctivitis, and fever. The mother states that the child has a rash that began from the face & spread caudally. The immunization record is not up-to-date. Physical examination is remarkable for a morbilliform rash over the body including the palms, and tiny grayish white dots on the buccal mucosa next to the third molar. What is the most likely Dx?

A) Rubella B) Mumps C) Measles D) Varicella

- A five yr old boy was brought to HC after having watery diarrhea of one day duration. He also has abdominal pain & vomiting of ingested matter, He drinks water eagerly. On examination, the child is irritable & has sunken eye balls & skin pinch goes back slowly. What is the most appropriate initial management for the pt?

A) Give fluid & food to treat diarrhea at home C) Give fluid for severe dehydration

B) Give fluid & food for some dehydration D) Give antibiotics for the infection

- 40. A four yr old girl was brought to a HC with complaints of cough, fever, & shortness of breath of two weeks duration. She has weight loss & loss of appetite. She was given Amoxicillin syrup a week back for the same complaint but she has no significant improvement. Her father has been coughing for the past six months. She is fully vaccinated. Her vital signs & anthropometry are in the normal range. Her ESR is 80 mm/hr. What is the most likely diagnosis?
- A) Bronchitis
- B) Pneumonia
- C) Presumptive tuberculosis
- D) Hyperactive airway disease

- An eight yr old child came to an OPD complaining of intermittent fever of one day duration. He also has repeated vomiting of ingested matter, one episode of generalized abnormal body movement & reddish discoloration of urine. On examination, his temp. is 39.6oc , GCS is 14/15 has pale conjunctiva with icteric sclera. A laboratory investigation result shows Hematocrite level of 28%, RBS of 45mg/dl & serum creatinine level of 1.1 mg/dl. (NR Serum Cr.= 0.6-1.2 mg/dl). What is the most prevalent severity sign in this child?

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| A) Jaundice | C) Renal failure |
| B) Convulsion | D) Haemoglobinuria |

- 45. A four yr old child was presented with complaints of barking cough, hoarseness of voice & inspiratory stridor. He was fully vaccinated. On physical examination. The child is cyanotic & in respiratory distress, his temp. is 38.5 oc , has stridor at rest & pharynx was erythematous. What is the initial Rx for this child?
- A) Dexamethazone
- B) Use of coal mist
- C) Air way management
- D) Nebulized racemic Epinephrine

- 46. A 10 month old boy was brought to an emergency OPD after having abnormal body movement which is characterized by flexion & extension of extremities, drooling of saliva & up rolling of the eyes that lasted for approximately three minutes. He was well except that he had fever preceding the abnormal body movement. His body temp is 39°C. Laboratory investigations are non-revealing. What is most appropriate next step in the management of this child?
- A) Diazepam
- B) Antipyretics
- C) Antiepileptic Drugs
- D) Reassure & send home

1. A 12 month old infant, who was on formula feeding since one month, was brought to the OPD for routine checkup. The mother reported that she has been feeding her child goat's milk. Vital signs were in the normal range. On CBC test, Hgb is 9.8g/l with MCV of 114.5ft, & Neutrophils are large & have hyper-segmented nuclei. What is the most likely explanation for this finding?

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| A) | β- thalasemia | C) Siderblastic anemia |
| B) | Iron deficiency anemia | D) Folate deficiency anemia |

- 48. A four yr old child was presented with complaint of vomiting & diarrhea of three days duration. On physical examination, vital signs are in the normal range, weight for height is 60%, & there is bilateral pitting edema. What is the most likely Dx?
- A) Marasmus
- B) Kwashiorkor
- C) Moderate malnutrition
- D) Marasmic-Kwashiorkor

- 49. A 5 yr old male child who was admitted in the ward & being treated for severe acute malnutrition (marasmus) for the past one week, currently developed profuse watery diarrhea of 12 hours duration. On physical he is lethargic with PR of 120/min, weak hair broken eye bal & skin pinch goes back slowly. What is the most appropriate next step in the management of this child?
- A) ResoMal 10 ml/kg over the first hour C) RL IV 15ml/kg over the 3rd hour
- B) RL IV 20m/kg over 30 minute D) RL 3ml/kg in 30 minute for b two hours.

- 51. A seven year old girl who is known diabetic pt. & is on Insulin, presented with complaints of abdominal pain & vomiting of two days duration. She also has increased frequency of micturition & thirst. She is confused & disoriented to time & place but her vital signs are in the normal range. Treatment was initiated immediately & after two hours her RBS becomes 170 mg/dl, & venous PH becomes 7.1 What is the most appropriate next step in the management of the pt.
- A) Add 5% Dextrose to fluid
- B) Increase same dose of Insulin
- C) Change IV fluids to oral
- D) Discontinue the Insulin

- A five yr old child was brought a pediatric OPD with a complaint of ear pain and low grade intermittent fever of four days duration. On physical examination, temp is 37.9 oc but other vital signs were in the normal range, Otoscopic examination revealed a bulged and hyperemic membrane, and minimal middle ear effusion. What is the next step in the management of the pt?
- A) Treat with oral antibiotics
- B) Send home with reassurance
- C) Advice to ear wash twice daily
- D) Admit and treat with IV antibiotics

A four-year-old child was brought by her parents with a complaint of whitish central spot on both eyes of one week duration. On history she was not vaccinated fully. She had history of maculopopular rash with runny nose and cough about two weeks back. On ocular examination. She has central corneal ulcer bilaterally.

- What is the most appropriate initial management for the child?
- A) Vitamin A supplementation C) investigation
- B) Topical antibiotics D) referral